Module 4

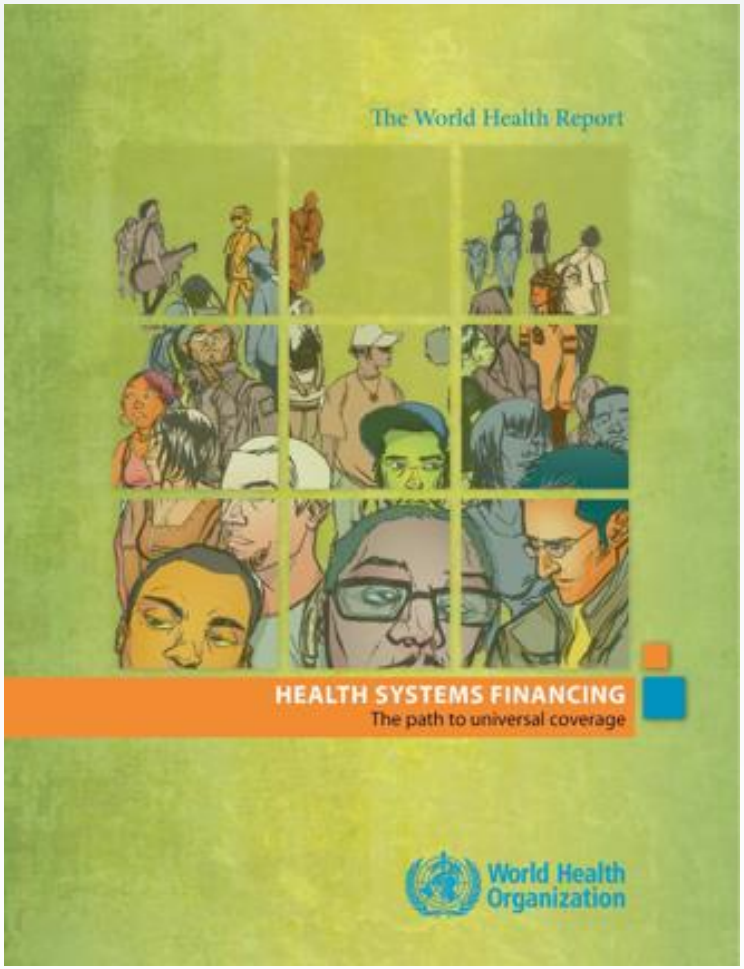
The future of Japan's health system of good health at low cost with equity

1. Introduction

A momentum towards universal health coverage

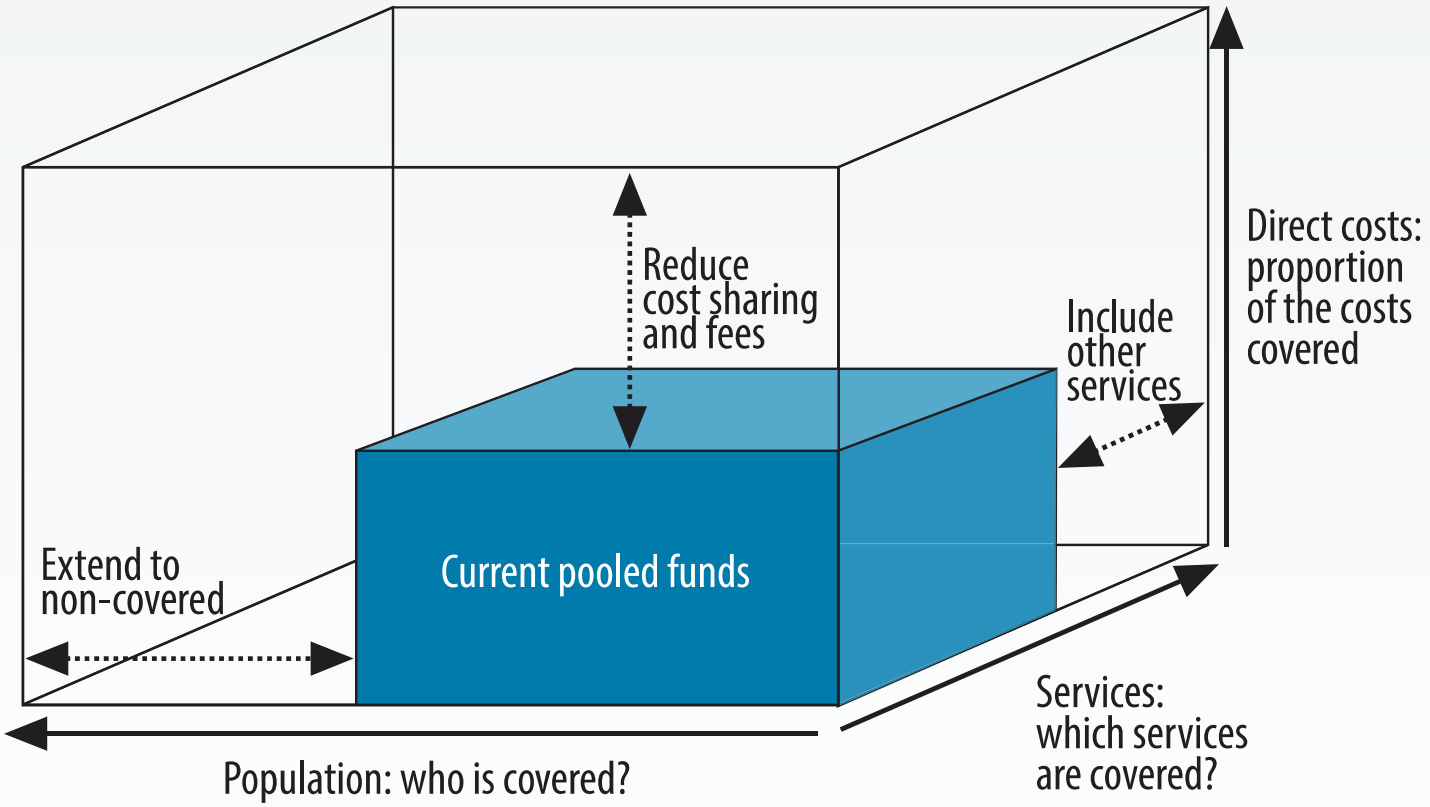
The World Health Report 2010
Health System Financing: The Path to Universal Coverage.

World Health Organization.
(2010).



Universal health coverage (UHC)

Three dimensions to consider when moving towards universal coverage



Source: World Health Organization (2010) The world health report: health systems financing: the path to universal coverage.

Japanese health system: good health at low cost with equity

1. An egalitarian system: universal coverage since 1961

2. Excellent population health (2013):

	Males	Females
Life expectancy at birth	80.1	86.4
Healthy life expectancy	71.1	75.6

3. Relatively low health care costs: 10.1% of GDP (2012)

Challenges

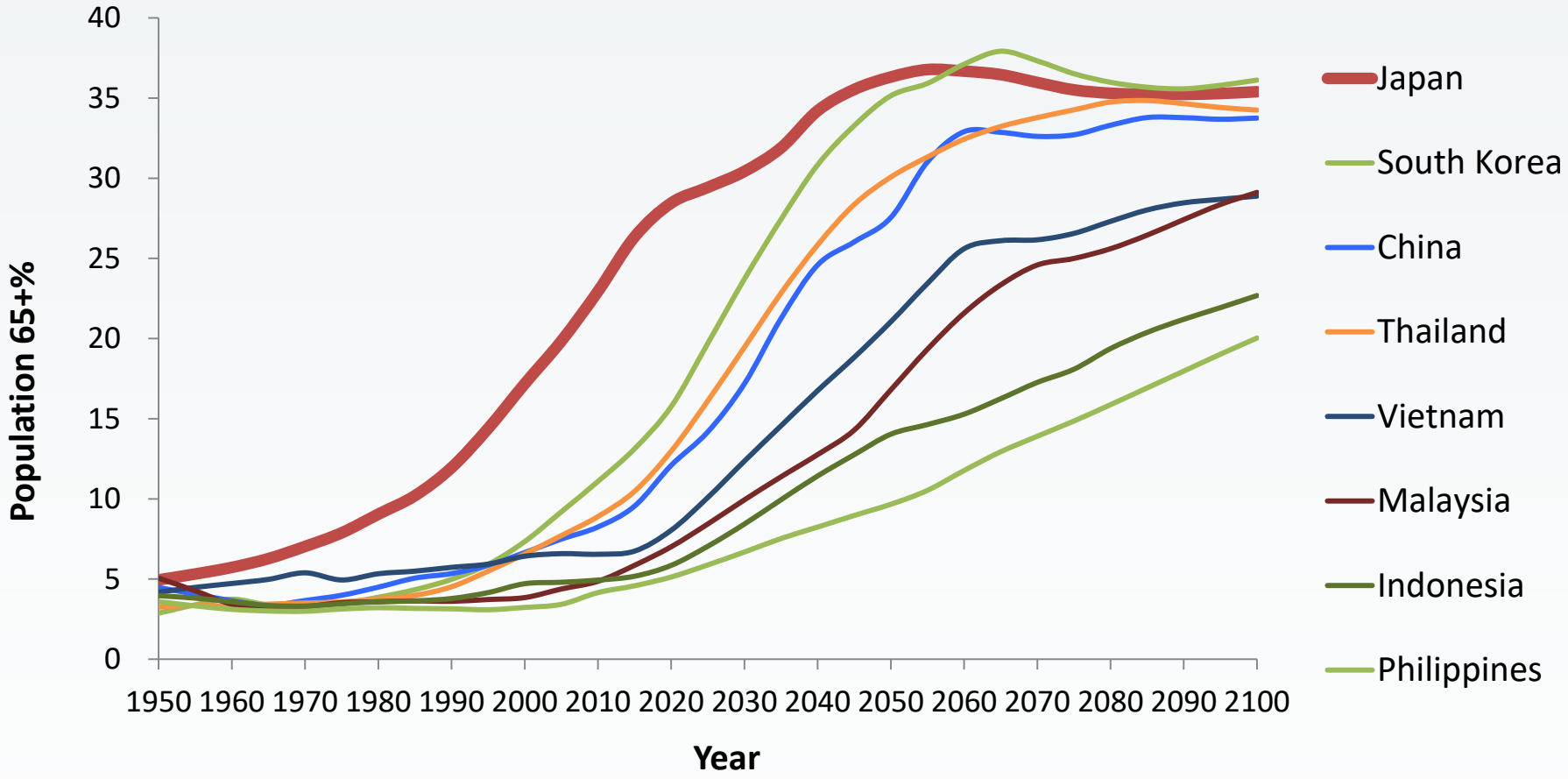
1. Access
2. Costs
3. Quality

How best to deliver value to patients?

“The incredible shrinking country”

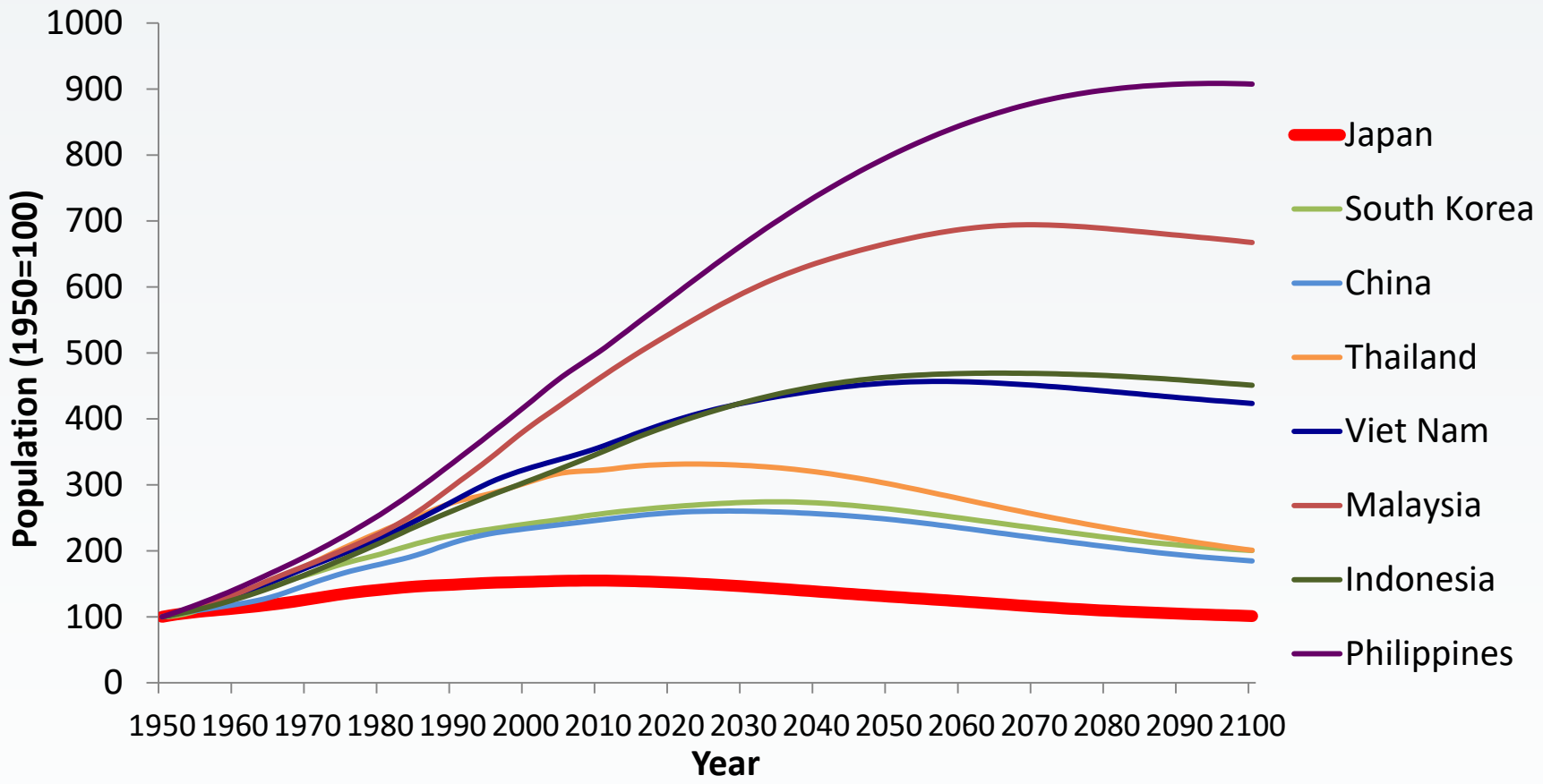
The Economist. Nov 20th, 2010

Proportion of population 65+ (%): 1950-2100



Source : United Nations, Department of Economic and Social Affairs, Population Division (2015a) World Population Prospects: The 2015 Revision

Population projections (1950 - 2100)



Sources : National Institute of Population and Social Security Research, Population Projections for Japan (January 2012) http://www.ipss.go.jp/site-ad/index_english/esuikei/gh2401e.asp

Lancet Series on Japan (2011): Universal Health Care at 50 Years

Source: Japan: Universal Health Care at 50 Years. The Lancet. Published: August 30, 2011

“Japan: a mirror for our future”

“The success of Japan’s health system matters not only because of its importance to Japanese citizens, but also because Japan is a barometer of western health...”

“The country now seeks to marshal its considerable resources to claim its rightful place globally, as well as to improve its policy making domestically.”

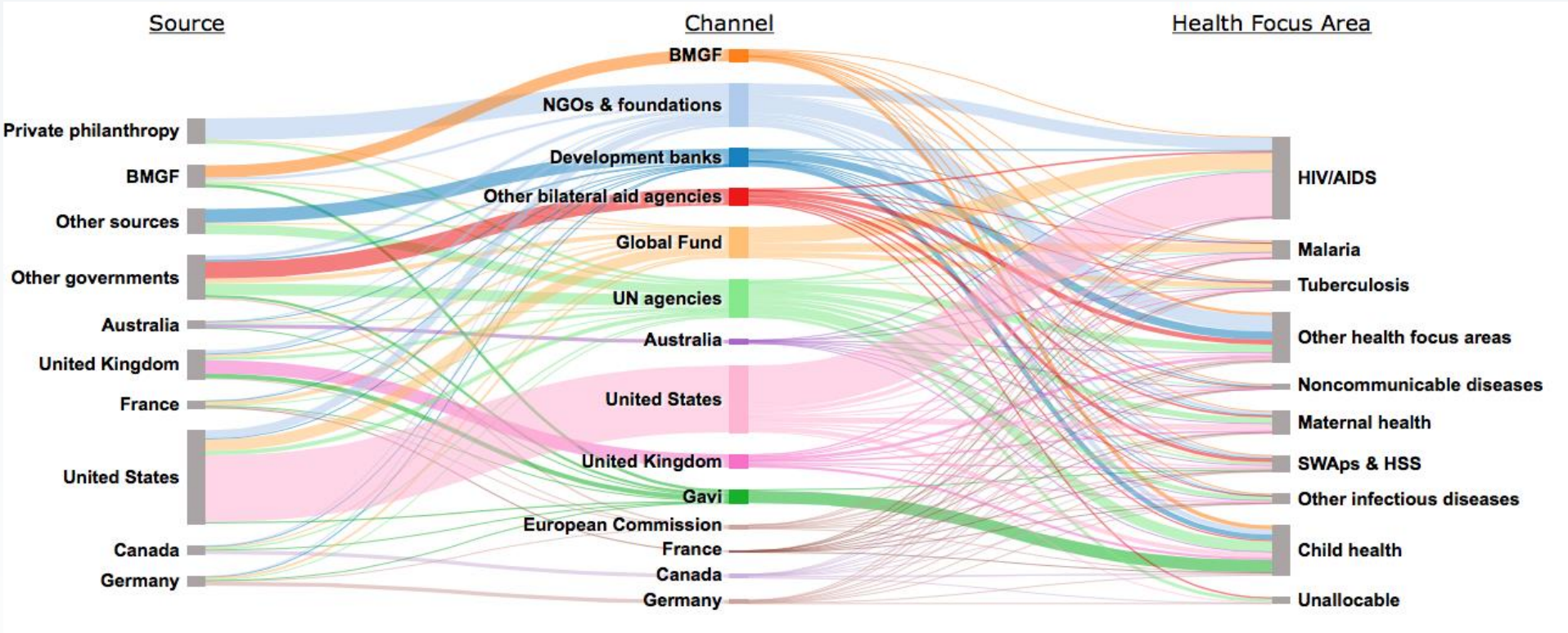
Source: Horton, Richard, Offline: Japan: a mirror for our future.
The Lancet , Volume 376 , Issue 9744 , 858

Summary

- Japan achieved UHC in 1961 which has in part contributed to good health at low cost with equity.
- This happened at the beginning of Japan's period of rapid economic development, while the country was still relatively poor (Japan's GDP was half of that in the UK).
- Now, Japan faces serious fiscal pressure due to rapid aging, low fertility and a sluggish economy and major challenges to sustain its health system.

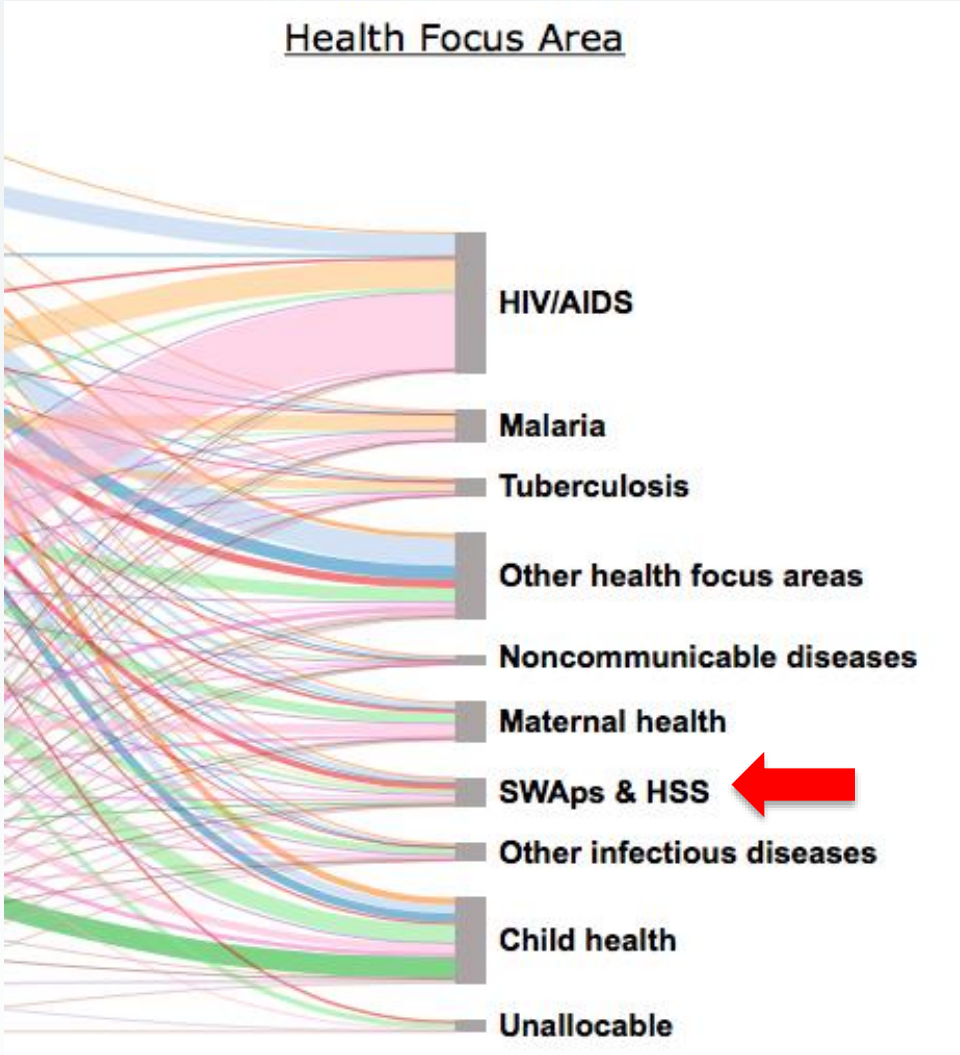
2. Understanding health system performance

Financing flow in global health, 2014



Source: Institute for Health Metrics and Evaluation (IHME). *Financing Global Health 2015: Development assistance steady on the path to new Global Goals*. Seattle, WA: IHME, 2016.

Financing flow in global health, 2014



Source: Institute for Health Metrics and Evaluation (IHME). *Financing Global Health 2015: Development assistance steady on the path to new Global Goals*. Seattle, WA: IHME, 2016.

Toward health system strengthening

"What is our key mission? I see WHO's role as being the moral voice and the technical leader in improving health of the people of the world. Ready and able to give advice on the key issues that can unleash development and alleviate suffering."

"I see our purpose to be combating disease and ill-health – promoting sustainable and equitable health systems in all countries"

Dr Gro Harlem Brundtland, WHO Director-General
Acceptance speech, May 1998
http://www.who.int/dg/brundtland/bruntland/en/

Defining health systems (1)

Many definitions in the literature:

- WHO's *World Health Report 2000*
- Harvard group 2004
- WHO *Everybody's Business 2007*
- OECD framework
- World Bank's 2007 report on healthy development

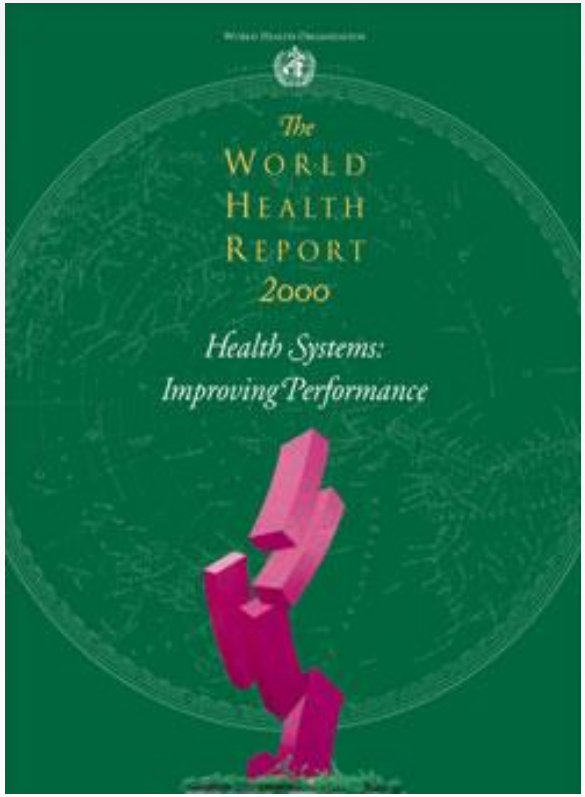
Defining health systems (2)

Some use the term system to refer to organized integrated care, others use the term system to define the range of actors with similar objectives.

WHO definition of health systems proposed in the *World Health Report 2000*:

- All actors, institutions and resources that undertake health actions.
- Health actions are all actions whose <u>primary intent</u> is to improve health.

World Health Report 2000



World Health Organization. (2000). *The World Health Report 2000: Health Systems: Improving Performance*.

- A landmark report to set an agenda for health systems.
- A primary focus on metrics for health system performance assessment.
- Limited descriptions of causes and cures for inadequate performance.
- The ranking of national health systems caused huge debates and controversies.

Key questions on health system performance

1. How much of the variation in health outcomes is due to differences in health system performance?
2. How much can the improvement of health system performance improve health outcomes in the future?
3. Which health system is doing a better job of improving health outcomes?
4. Which health system is getting value for money?
5. What are the determinants of health system performance?

Source: Murray, C. J., & Frenk, J. (2000). A framework for assessing the performance of health systems. *Bulletin of the World Health Organization*, *78*(6), 717–731.

Framework for health system performance assessment (1)

1. A simplified input-output framework
2. Inputs to the system (functions):
   - Financing (revenue collection, pooling and purchasing)
   - Resource generation (human resources, facilities and equipment, knowledge, etc.)
   - Stewardship (broader than the conventional notion of regulation)
   - Service provision (personal and non-personal services)

Source: Murray, C. J., & Frenk, J. (2000). A framework for assessing the performance of health systems. *Bulletin of the World Health Organization*, *78*(6), 717–731.

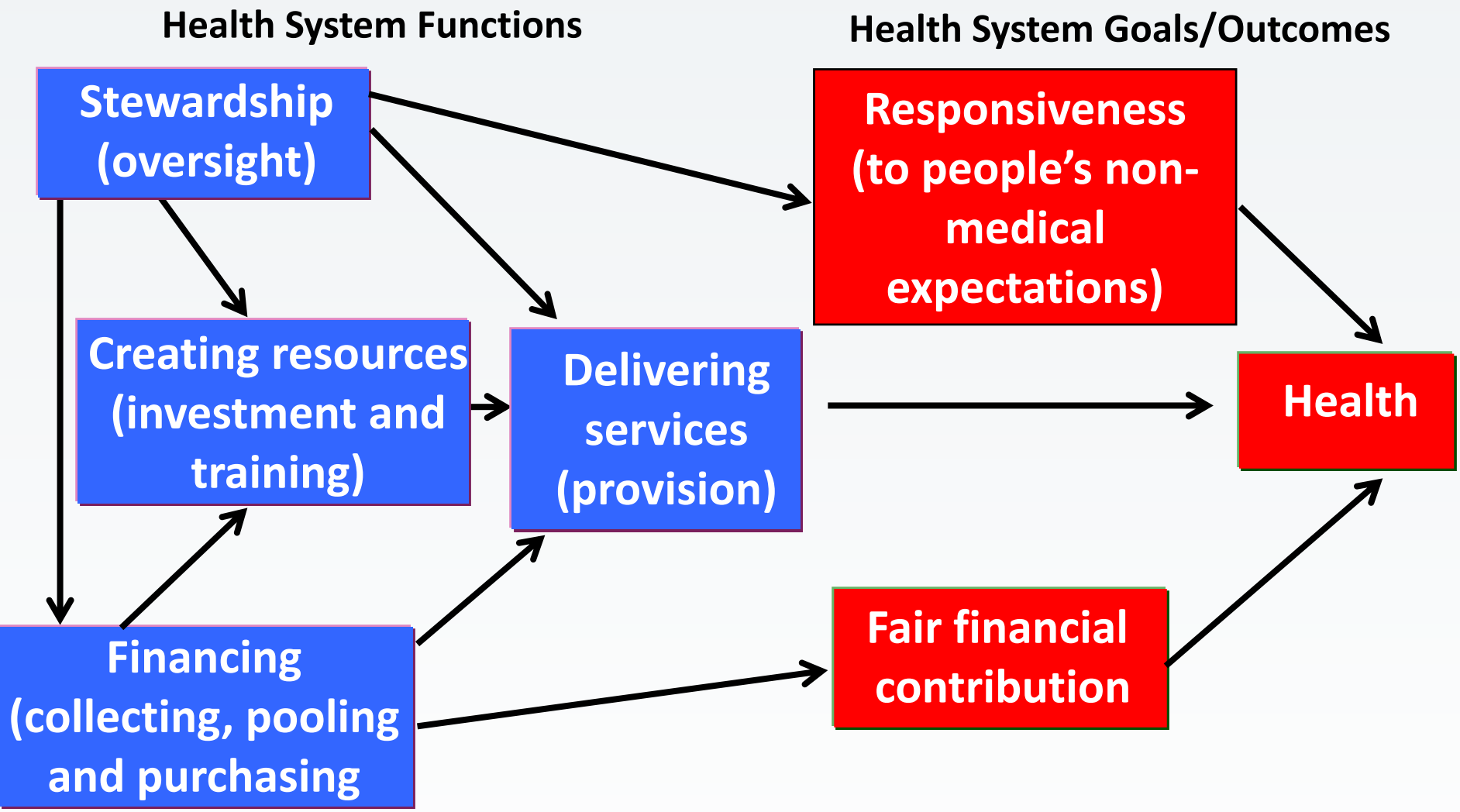
Framework for health system performance assessment (2)

3. Health system goals/outcomes:

- Health (average and distributions)
- Responsiveness (dignity, confidentiality, prompt attention, amenities, etc.)
- Fairness in financial contribution

Source: Murray, C. J., & Frenk, J. (2000). A framework for assessing the performance of health systems. *Bulletin of the World Health Organization*, *78*(6), 717–731.

HSPA framework



Source: Evans, D. (2003). Health systems performance assessment: debates, methods and empiricism. *Geneva: World Health Organization*.

Health system goals

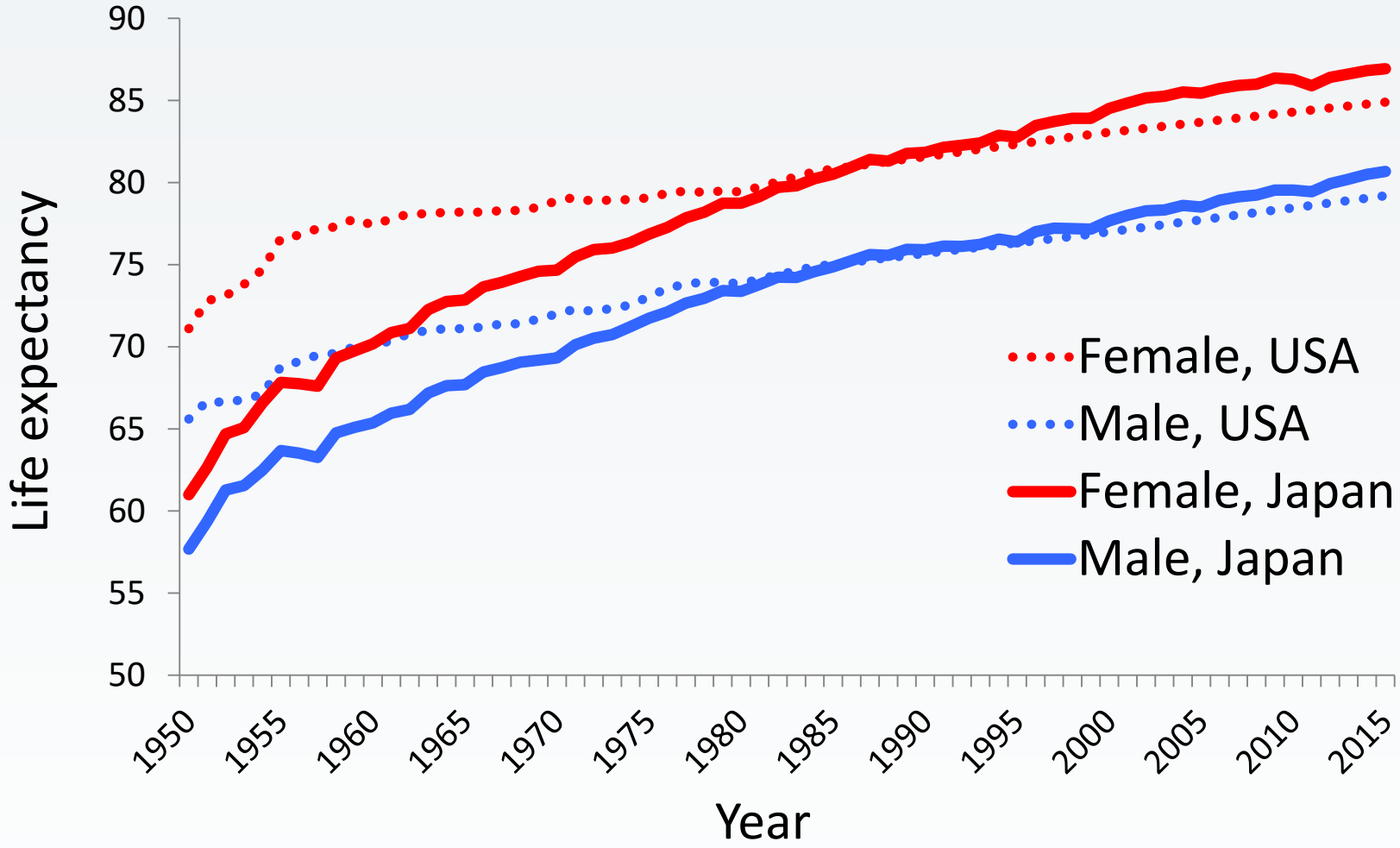
		Average	Distribution
Efficiency	Health improvement	X	X
	Responsiveness to expectation	X	X
	Fairness in financial contribution		X
		Quality	Equity

Summary: health system performance

1. There is growing interest in health system strengthening but limited investment.
2. Disease control still dominates global health activities.
3. Monitoring and evaluation based on a clear conceptual framework is the key to understanding the complexities of health system strengthening.

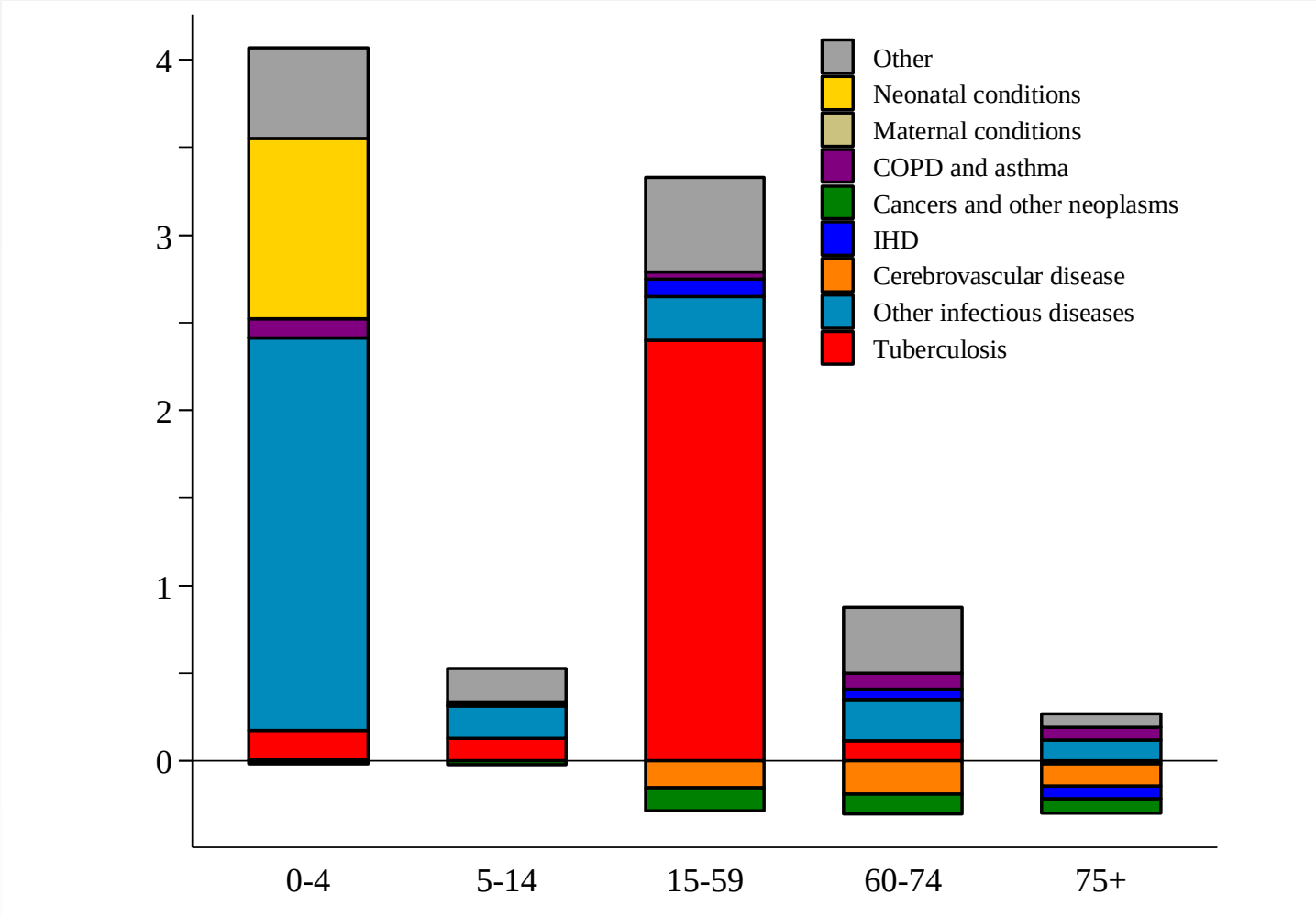
3. Population health

Comparative life expectancy, Japan and USA, 1950-2015



Source: Japan National Institute of Population and Social Security Research, US Census Bureau

Age- and cause-specific contributions to changes in life expectancy at birth in Japan: 1950–1965 (males)



Source: Ikeda, N., Saito, E., Kondo, N., Inoue, M., Ikeda, S., Satoh, T., ... & Noda, M. (2011). What has made the population of Japan healthy?. *The Lancet*, *378*(9796), 1094-1105.

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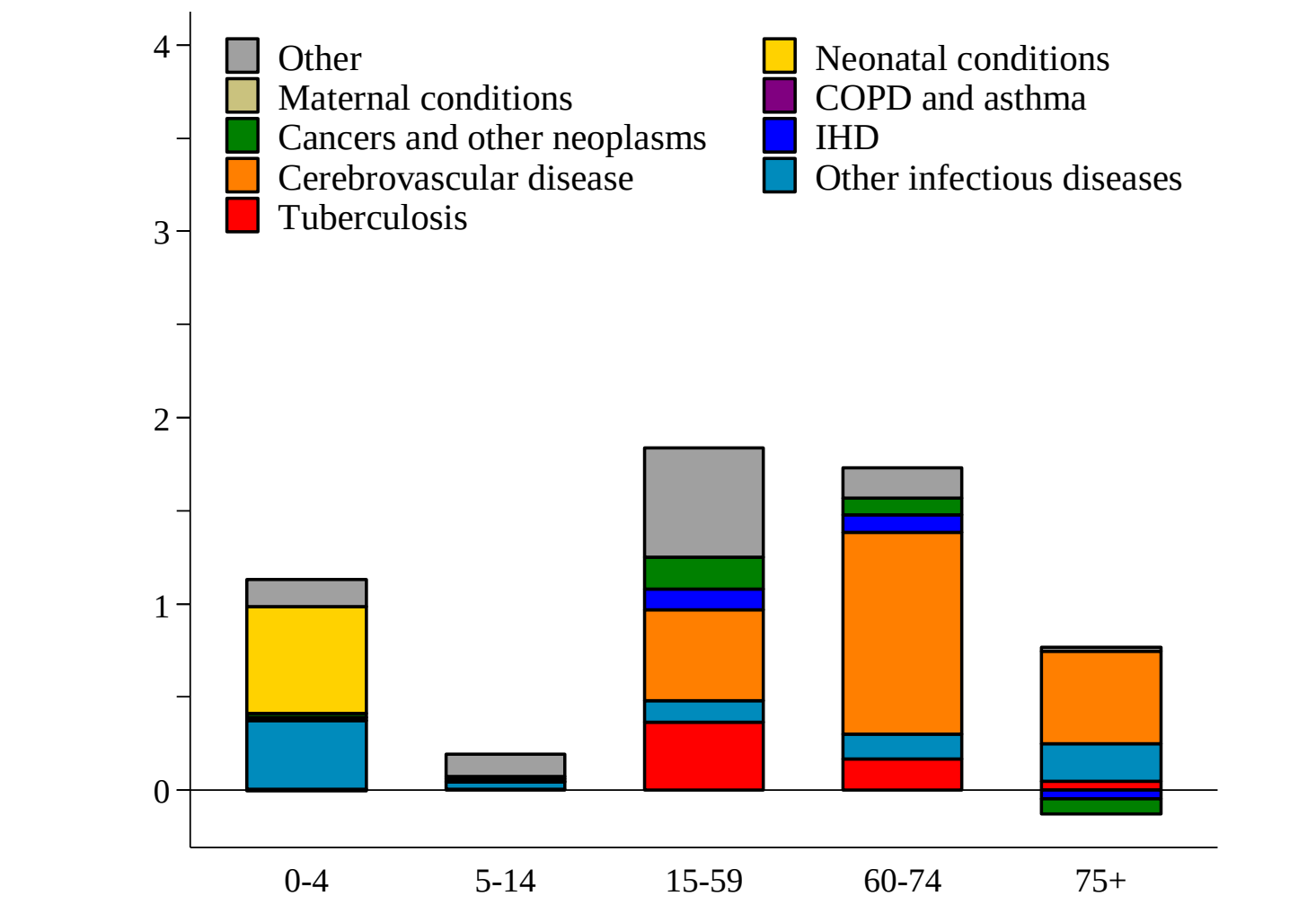

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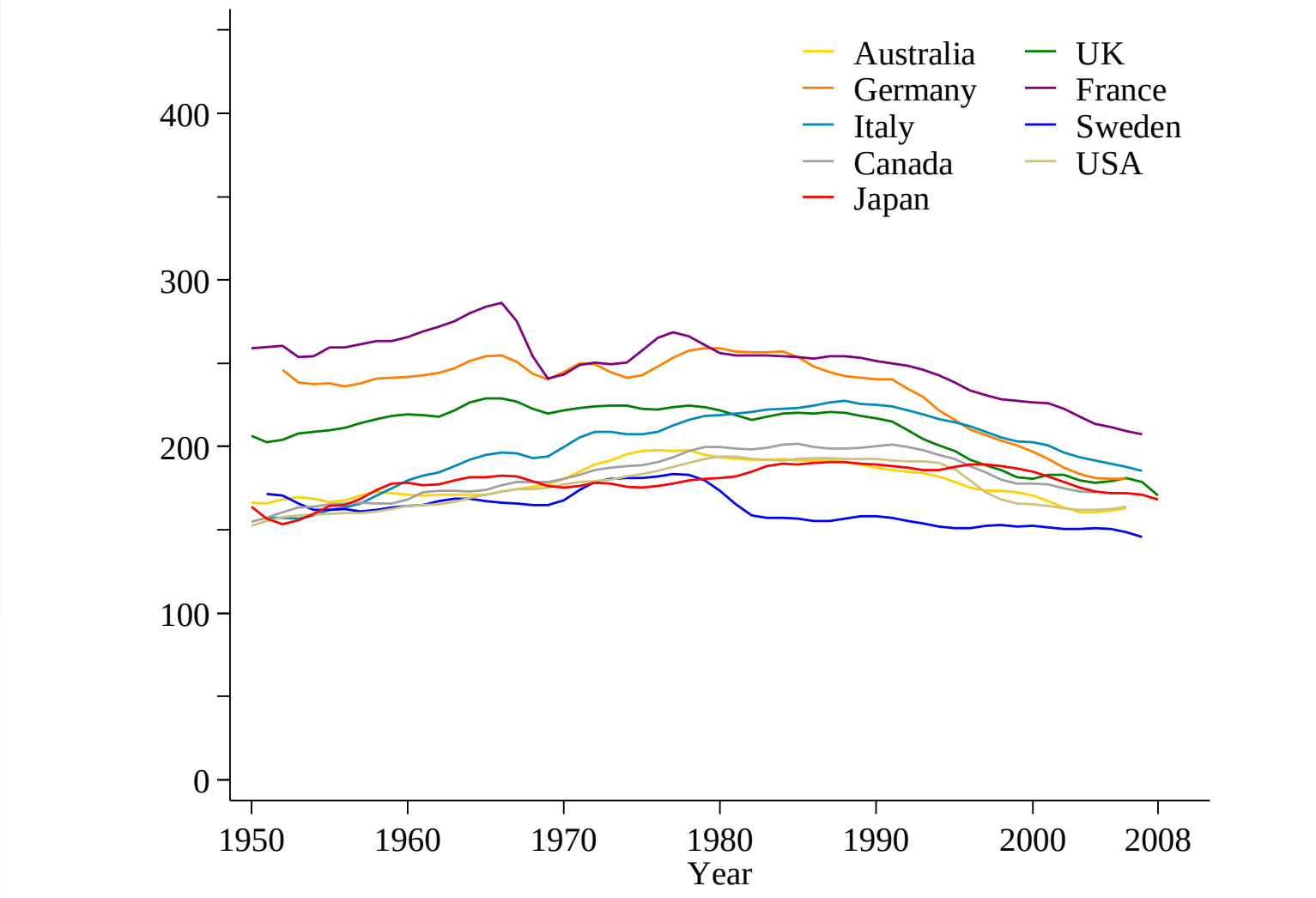
Age-standardized cancer mortality (males)



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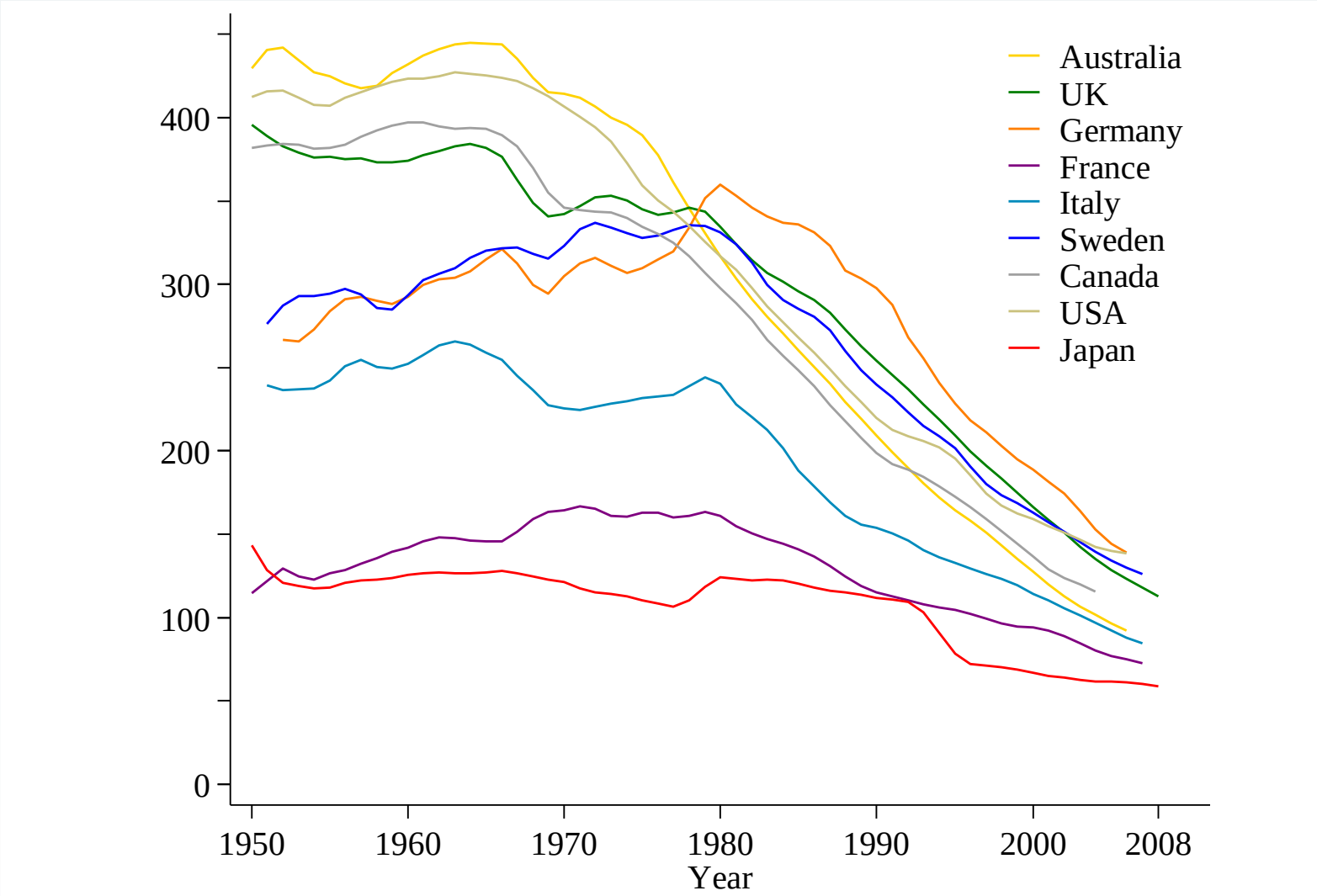
Age-standardized ischemic heart disease mortality (males)



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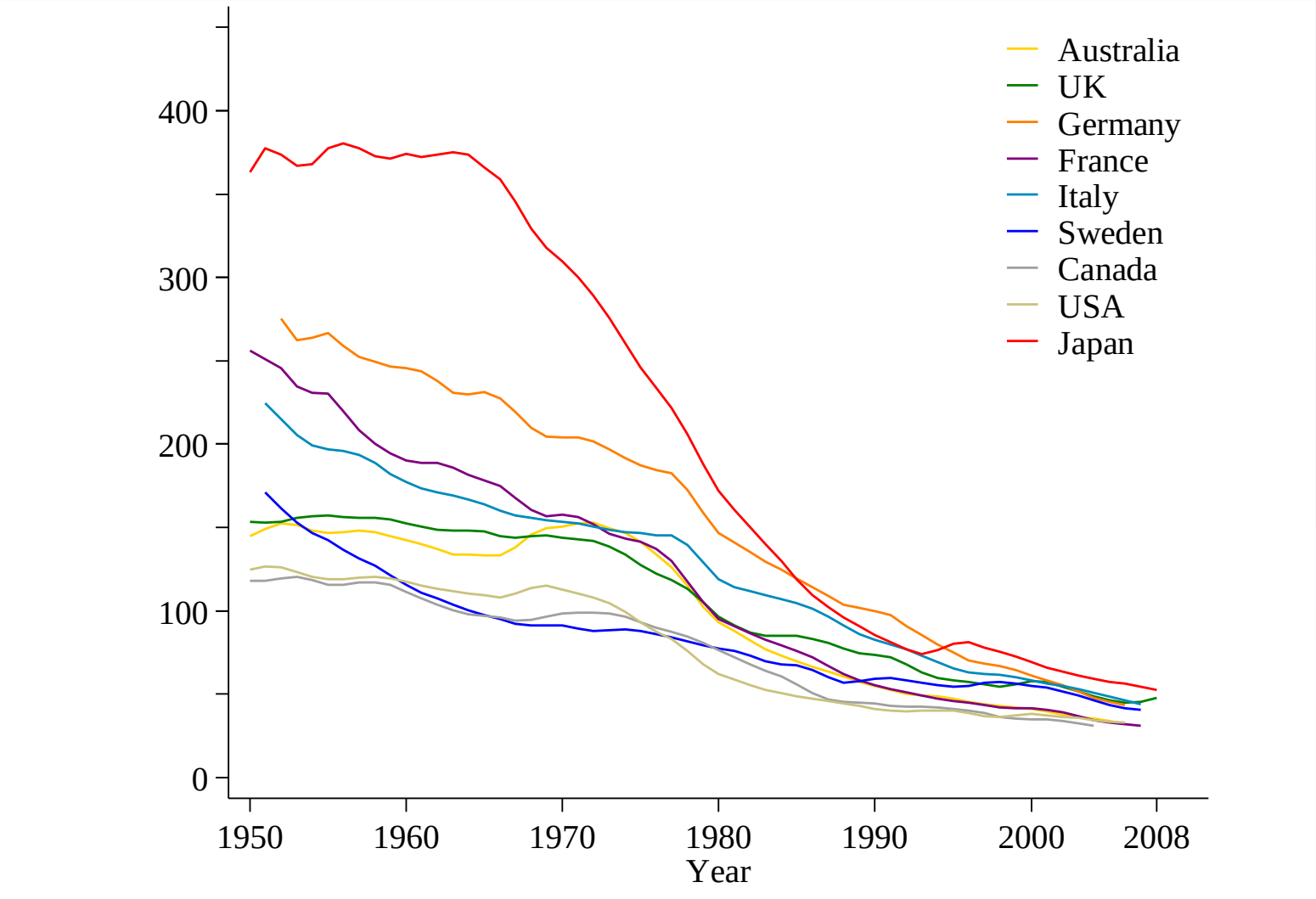
Age-standardized stroke mortality (males)



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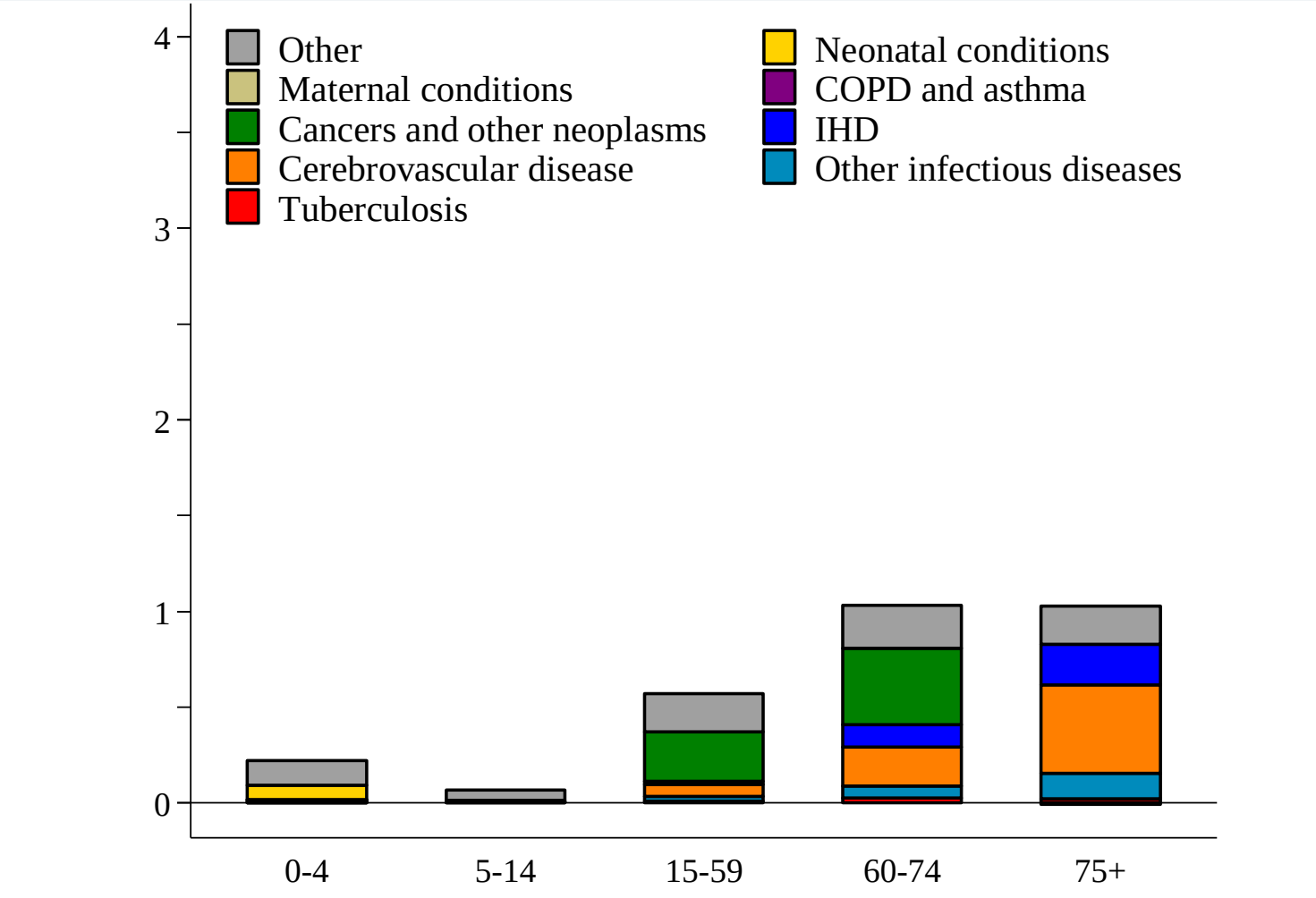
Age- and cause-specific contributions to changes in life expectancy at birth in Japan: 1995–2008 (males)



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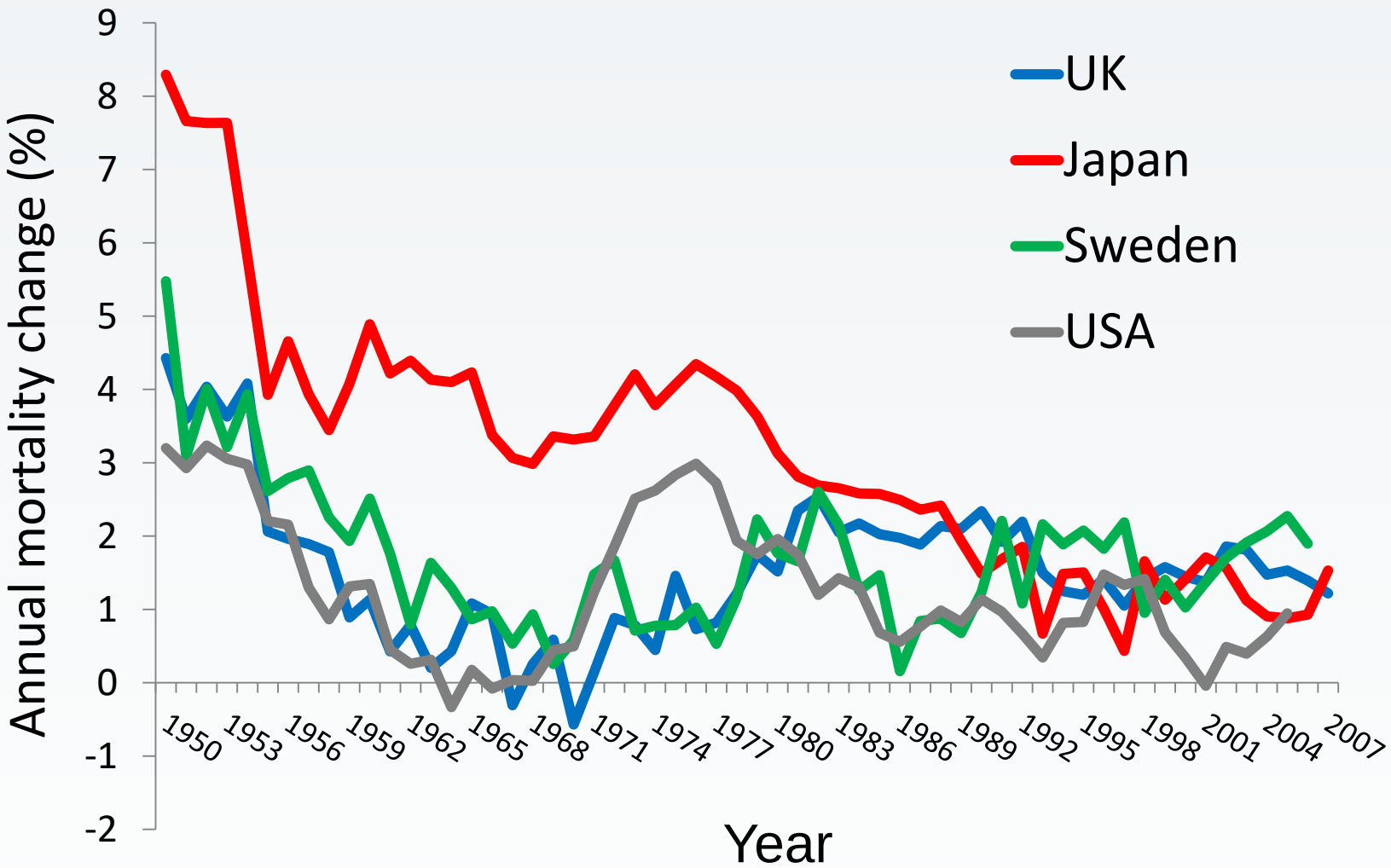
Burden of disease and risk factors in Japan (2013)

Rank	Deaths	Burden of disease (DALYs)	
		Causes	Risk factors
1	Cerebrovascular disease	Low back and neck pain	Dietary risks
2	Lower respiratory infections	Cerebrovascular disease	High blood pressure
3	Ischemic heart disease	Ischemic heart disease	Tobacco smoke
4	Lung cancer	Lower respiratory infections	High fasting glucose
5	Stomach cancer	Lung cancer	Alcohol and drug use
6	COPD	Other musculoskeletal	High BMI
7	Colorectal cancer	Diabetes	Low physical activity
8	Alzheimer disease	Sense organ diseases	Low GFR
9	Liver cancer	Depressive disorders	Air pollution
10	Chronic kidney disease	Self-harm	High total cholesterol

Source: Global Burden of Disease Study 2013 (GBD 2013) Data Downloads
http://ghdx.healthdata.org/global-burden-disease-study-2013-gbd-2013-data-downloads, ODC-BY

Annual change in adult mortality rate (females)



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Summary: population health

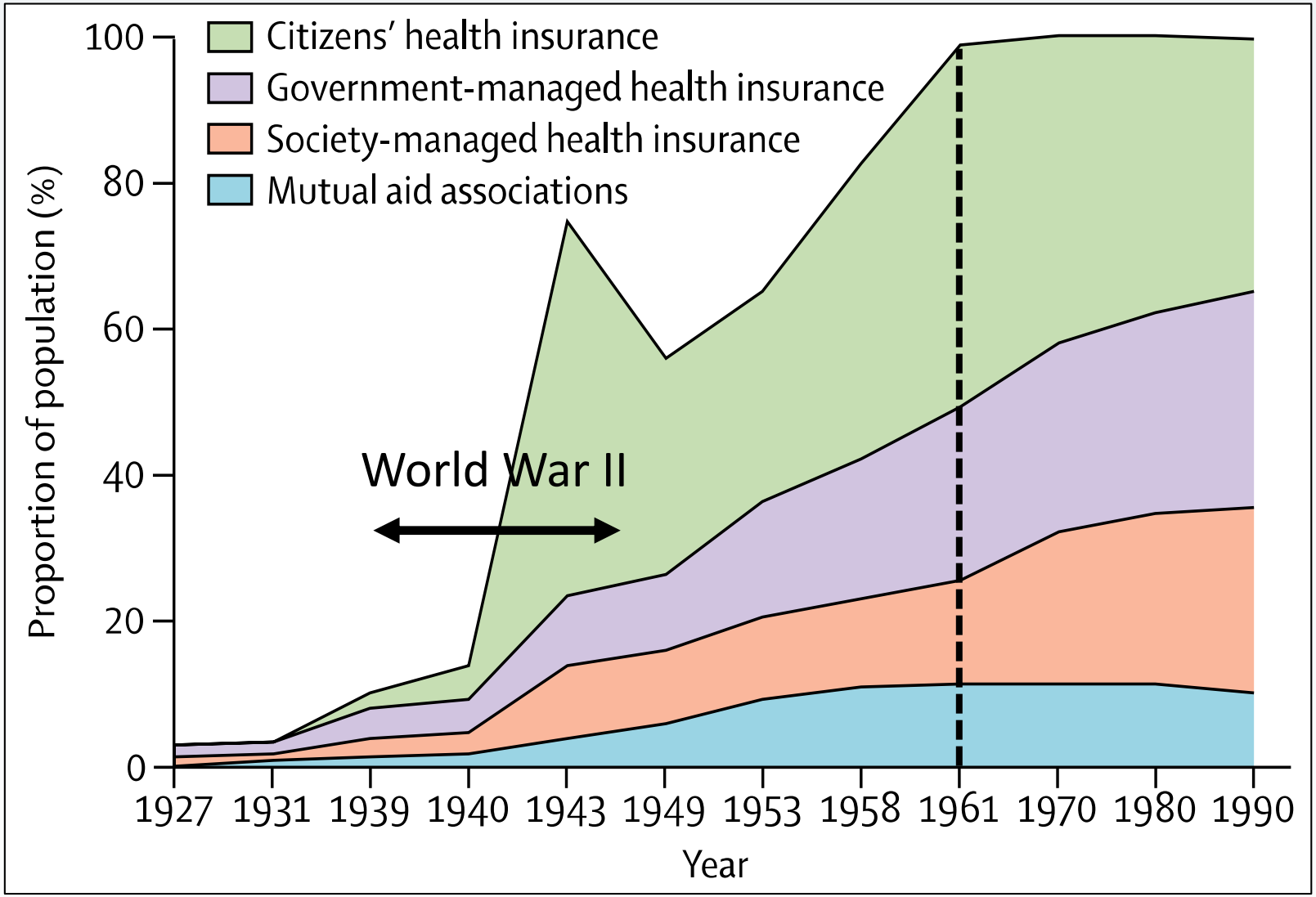
1. Strong government action can achieve effective infectious disease control even at low national income in an educated population
2. Main effects of the health-care system on mortality through public health action, primary care management of key risks (e.g. blood pressure)
3. Japan benefited from favorable risk factors (e.g. for ischemic heart disease and some cancers)
4. In an era of economic stagnation, political turmoil, ageing populations, inadequate tobacco control, Japan is ineffective in addressing these new challenges

4. UHC in Japan: achievements and challenges

Status of social health insurance plans in Japan

- Individuals have no choice of plans. Dependents (under 75 years) are covered by household plan
- Proliferation of plans (about 3500)
- Half of plans employee-based, half community-based
- All plans cover same services, including drugs and dentistry
- Payments to providers are the same in all plans
- High co-payment rate, but proportion paid by patients only 14% of national medical expenditure

A path toward UHC



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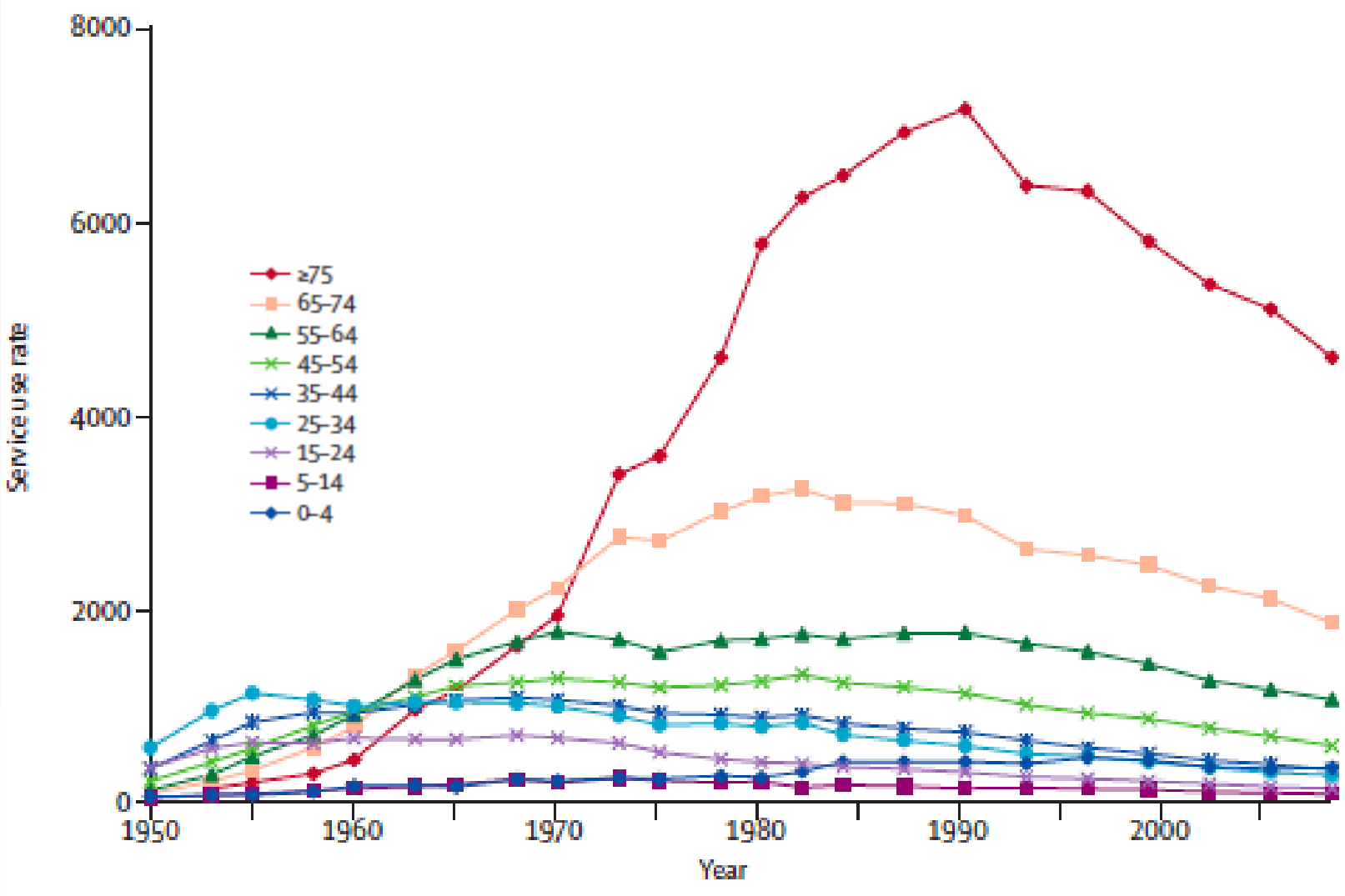
Trends in inpatient service usage rate by age



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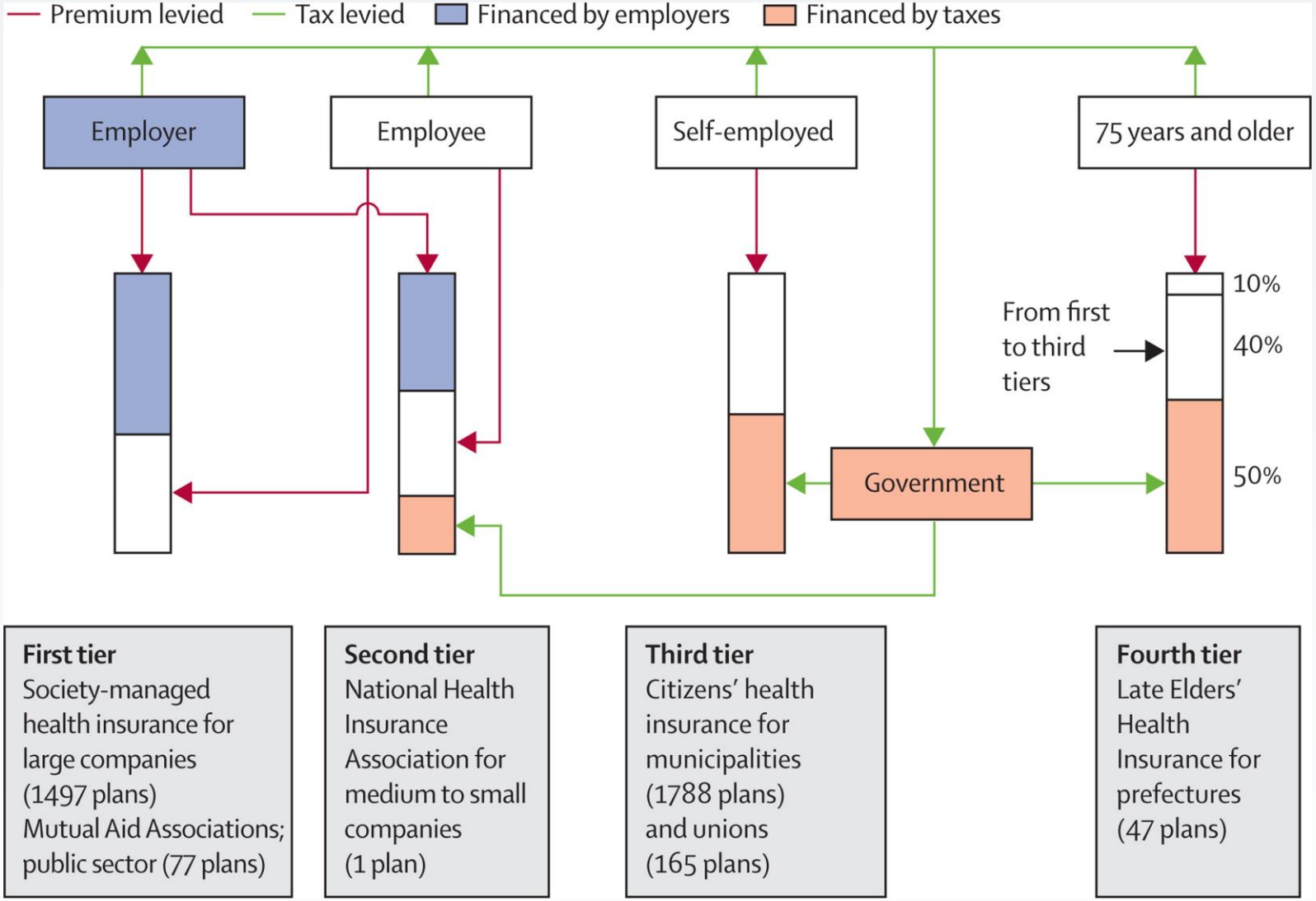
Subsidies and cross-subsidization



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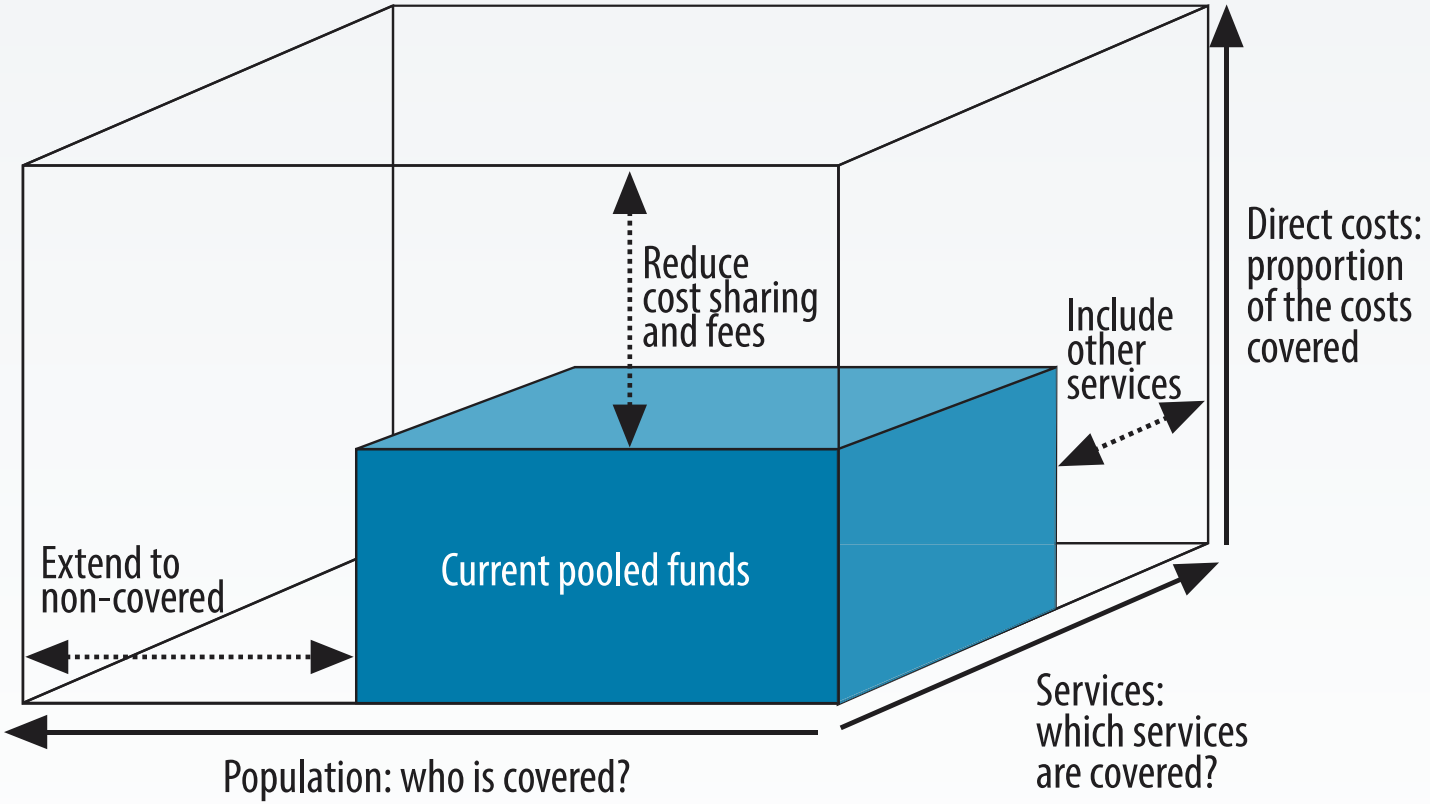
Universal health coverage (UHC)

Three dimensions to consider when moving towards universal coverage



Source: World Health Organization (2010) The world health report: health systems financing: the path to universal coverage.

Extent of equity

1. Horizontal equity
2. Fairness in financial contribution
3. Prevention of catastrophic payment

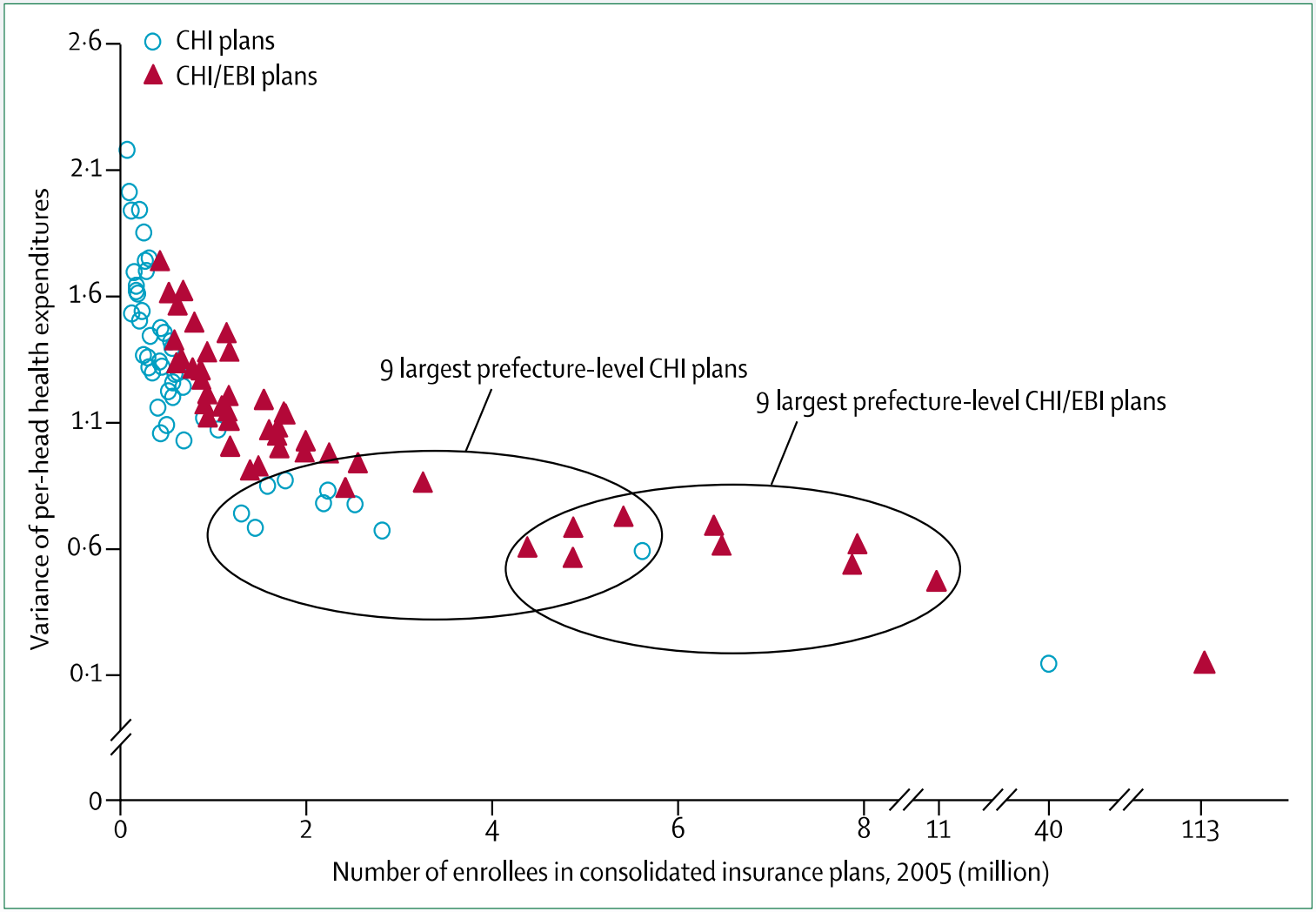
Challenges to the sustainability of UHC

1. Population aging
2. Change in working patterns
3. Uninsured

Financing options to sustain the current system

1. Increase tax
2. Increase premium
3. Reduce benefit package

Impact of consolidation



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Summary: UHC in Japan

1. Japan successfully expanded coverage, contained costs, improved equity.
2. Progress to universal coverage can begin while still low-income
3. Health expenditure can be controlled while expanding coverage and improving equity.
4. Incremental adjustments can appear rational but create future pressures.

5. Cost containment

A paradox of the Japanese health system

Many characteristics to increase in costs:

1. Private provision of services
2. Fee-for-service basis
3. No waiting lists/no gate-keeping: unrestricted access to virtually all hospitals
4. No limitations on the purchasing of expensive equipment (e.g. the highest per capita number of MRI)

Source: Hashimoto, H., et al. (2011). Cost containment and quality of care in Japan: is there a trade-off?. *The Lancet*, *378*(9797), 1174-1182

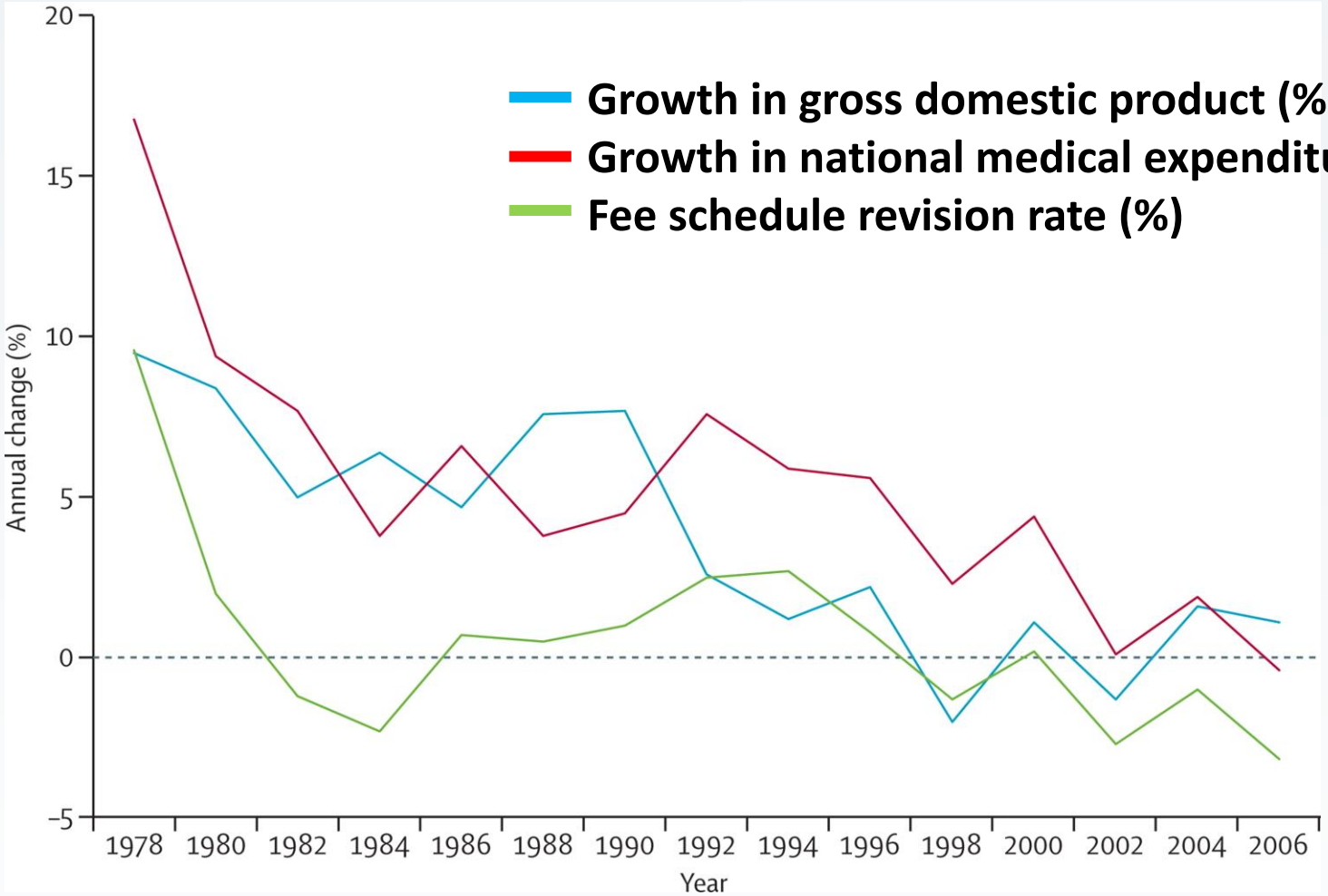
How does Japan manage to keep costs low?

"Fee schedule" determines both prices and conditions for payments.

1. Supply-side control
2. Single payment system with multiple payers (over 3500 payers)
   - Efficient: no negotiation needed on an individual basis
   - Equitable: the benefit package is essentially the same for all social health insurance plans
   - Easier for price control
   - Effective for introducing new policies (possibly)

Source: Hashimoto, H., et al. (2011). Cost containment and quality of care in Japan: is there a trade-off?. *The Lancet*, *378*(9797), 1174-1182
Ikegami, N., et al. (2011). Japanese universal health coverage: evolution, achievements, and challenges. *The Lancet*,*378*(9796), 1106-1115.

Impact of the revision of fee schedule on health expenditures



Annual changes in gross domestic product, national medical expenditure, and global revision rate of the fee schedule in Japan, 1978–2006

Source: Hashimoto, H., et al. (2011). Cost containment and quality of care in Japan: is there a trade-off?. *The Lancet*, *378*(9797), 1174-1182

Service provision in Japan

1. Laissez-faire
   - 80% of hospitals, 94% of clinics in private sector
   - Physicians can open clinics anywhere
   - Physician freedom in specialization
   - Hospital freedom in equipment purchasing, organization
2. Strict regulations
   - Revenue control by fee schedule
   - Legal prohibitions on investment/ownership
   - Profits must be reinvested in capital (non-profit hospitals)

Why didn't professionals protest the item-by-item price reduction?

1. Doctor protests were over-ruled.
2. Japan Medical Association: major ruling party donor.
3. Lack of specialist / hospital organization.
4. Disparity in specialist pay (urban hospitals vs. rural physicians).
5. This has enhanced access to care and contained costs

Problems with fee schedule

1. Lack of reasonable and transparent grounds for price setting, while relying on political negotiations under a macro cap.

2. Prices of outpatient services and pharmaceutical prescriptions have higher profit margins compared to inpatient acute care services such as surgery

3. Reflects role of the JMA, which has more delegates from clinics.

Source: Shibuya, K., et al. (2011). Future of Japan's system of good health at low cost with equity: beyond universal coverage. *The Lancet*,*378*(9798), 1265-1273.

Supply-side fee control becomes less effective

1. Fee cuts do not reduce demands
2. Prices can fall only so far before products become unavailable
3. The quality of care suffers
4. Natural annual increase of medical expenditures at 6% (3% each due to medical technology and aging)
5. Limitations of patchwork style of health policy making to maintain the status quo

Summary: cost containment

1. Uniform fee schedule can control drug and services prices
2. However reviews and inspections reduce physician incentives
3. Global shift from annual budgets towards payment by results
4. Organisational capacity required:
   - Setting fees
   - Monitoring regulatory adherence
   - Negotiating price revisions
   - Collecting data to analyse performance

6. Service quality

Hospitals and clinics in Japan

1. About 8,700 Hospitals
2. Two-tier system:
   - Large public hospitals for acute care
   - Small private clinics/hospitals for non-acute care (major supporters for JMA)
3. Relatively low number of physicians per capita
4. Poorly differentiated hospital function (acute, sub-acute, and chronic care)

Monitoring: focusing on inputs

- Number of beds and nurses per beds
- If the hospital does not meet basic standards, it faces sanctions through reduced reimbursement rates.
- The higher the nurse staffing level is, the higher the reimbursement rate will be.

Determinants of hospital-standardized mortality ratio in acute inpatient care in Japanese hospitals

	Regression coefficient, unstandardised	p>Z
Staffing (in full-time equivalent)		
Number of physicians	–0·300	<0·0001
Number of nurses	–0·022	0·4169
Number of pharmacists	–0·252	<0·0001
Structural input of hospital		
Number of subspecialty programmes	–0·149	<0·0001
Number of acute care beds	–0·120	0·0084
Number of beds in intensive care unit	–0·315	<0·0001
Other hospital characteristics		
Annual number of general anaesthesia procedures	–0·153	<0·0001
University main hospital	–0·018	0·8339
Presence of emergency service unit	–0·401	<0·0001
Proportion of acute care beds to total beds	–0·041	0·4471
Ratio of inpatient to outpatient service volume	–0·188	<0·0001

Source: Hashimoto, H., et al. (2011). Cost containment and quality of care in Japan: is there a trade-off?. *The Lancet*, *378*(9797), 1174-1182

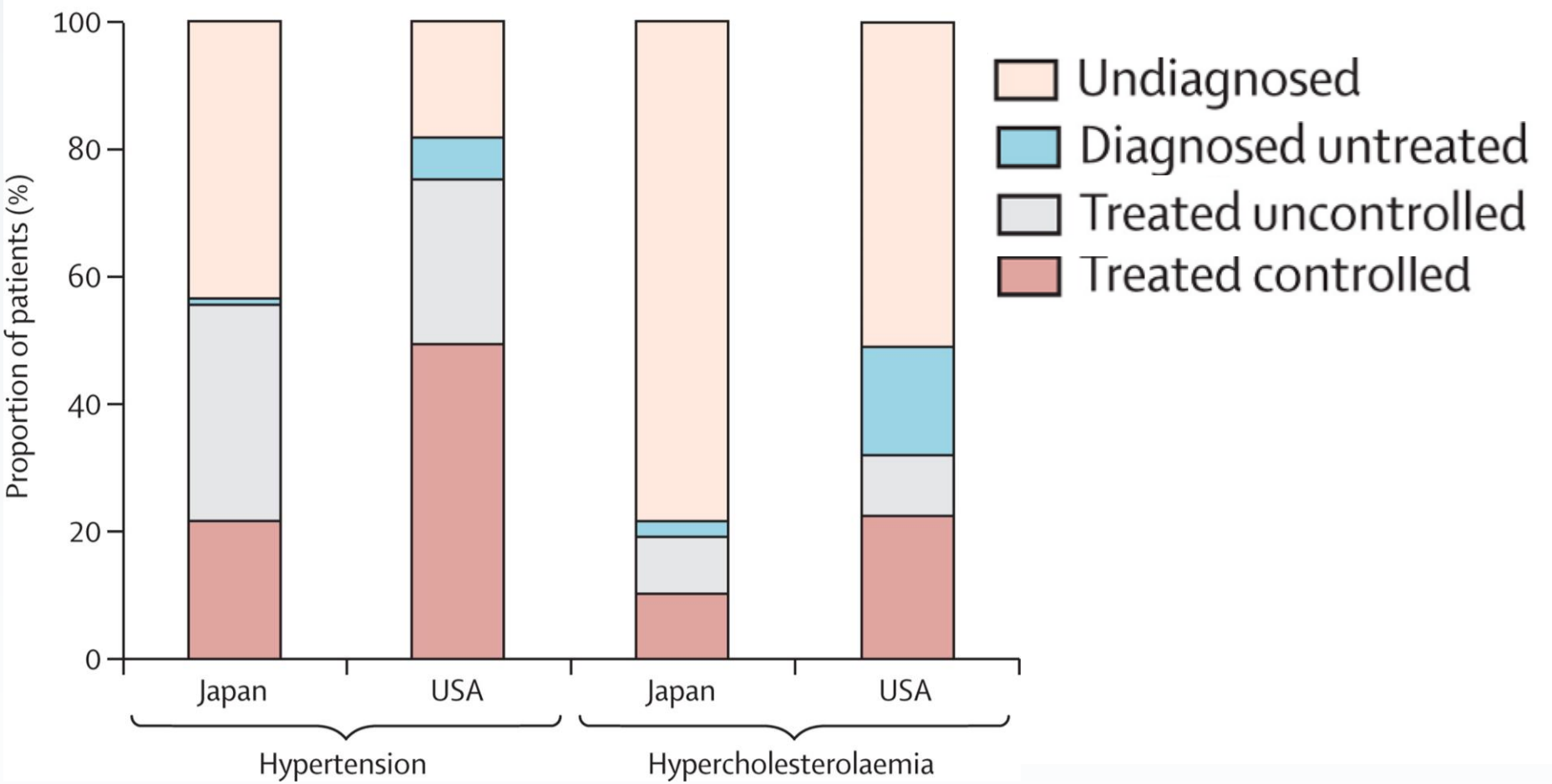
Structure of physicians and hospitals: consequences of fee schedule and medical education

Vertical physician relationships than horizontal institutional relationships

Antagonism between public / private hospitals

- Concentration of power in the JMA
- Other professional societies and hospital associations weaker
- Income gap between private practitioners and hospital specialists

Coverage and control of hypertension and hypercholesterolemia in Japan and the USA



Source: Hashimoto, H., et al. (2011). Cost containment and quality of care in Japan: is there a trade-off?. *The Lancet*, *378*(9797), 1174-1182

Challenges

1. Limited incentives to improve quality (e.g. outcome)
2. Demand-supply mismatch
3. Fragmented hospitals, poorly differentiated
4. Weak accreditation standards

Suggested reforms

- Increase flexibility on the payment side and tighten control of service delivery
- Strengthen capacity of prefectural governments to control the delivery of services
- Reform medical education focusing more on primary care

Summary: service quality

1. Excellent outcomes despite relatively poor structural and process dimensions of quality of care

2. Demand-supply mismatch and impeded accountability due to Japan's policy of tight control of health-care cost and a laissez-faire approach to service delivery

3. Initial priority on expansion of access and prevention of impoverishment from health care, followed by promoting efficiency and quality of services

7. Population aging

Tackling the aging population

- Early 1970s: “free medical care”, abolition of all elderly co-payments; “social admissions” by elderly people.
- 1989: Sudden expansion of government responsibility for frail elderly (“Gold Plan”).
- 1990s: The Gold Plan created finance, quality, management, governance, and HR issues.

Tackling the aging population

- Mid 1990s: Proposed reform of long-term care through social insurance (LTCI).
- 2000: Second decision to expand government responsibility for frail older people ("Gold Plan").

Long-Term Care Insurance in Japan

- Premiums paid for all aged 40+.
- Benefit rights determined by physical /mental condition
- Independent of income or family care.
- Eligibility determined objectively (assessment questionnaire)
- Recipients choose services and providers.
- Formal services only (no cash allowance for informal care)

Source: Tamiya, N., et al. (2011). Population ageing and wellbeing: lessons from Japan's long-term care insurance policy. *The Lancet*, *378*(9797), 1183-1192.

Impact of long-term care insurance

1. Wellbeing of care recipients
2. Wellbeing and opportunity losses of caregivers
3. Household economy

Source: Tamiya, N., et al. (2011). Population ageing and wellbeing: lessons from Japan's long-term care insurance policy. *The Lancet*, *378*(9797), 1183-1192.

Achievements of long-term care insurance

1. Universal coverage
2. Increased use of formal care and reduced financial burdens
3. Potential system for evidence-based planning, monitoring and evaluation

Source: Tamiya, N., et al. (2011). Population ageing and wellbeing: lessons from Japan's long-term care insurance policy. *The Lancet*, *378*(9797), 1183-1192.

Successes and challenges in long-term care insurance (LTCI)

1. LTCI services and expenditures doubled in ten years
   - Health expenditures increased by only 15%
   - Acceleration of increase projected for 2010
   - LTCI fee schedule raised by 3% in 2009
2. Fee schedule for LTCI may not be as effective as in health care.
3. Possible solution: narrow eligibility criteria, decrease benefit amounts
   - Politically easier in LTC than in healthcare?
   - Government's responsibility decent, not best, level of care

Summary: Japan's long-term care insurance

1. Public LTCI introduced because of rapid aging, failures in medical / social service systems.
2. Generous entitled benefits based on explicit eligibility criteria.
3. New policy can be introduced when opportune and adjustments made later.
4. Government role in LTC raises basic questions about social values and economic sustainability.

8. Future directions

Provisional Translation

保健医療 2035
JAPAN VISION: HEALTH CARE

Japan 2035

Leading the World through Health

The Japan Vision: Health Care 2035

Brief Summary

http://www.mhlw.go.jp/healthcare2035

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Source: Health Care 2035 Advisory Panel (2015) The Japan Vision: Health Care 2035 Summary. http://www.mhlw.go.jp/healthcare2035

Japan 2035

Leading the World through Health

A health care system built for the next 20 years and designed for all lifestyles and people – from children to older people, from patients to providers – where individuals feel secure and supported to make the life and work choices that are right for them.

Amidst rapid population ageing, advances in medical technology, and major shifts in health care, a health care system that contributes to financial stability while engaging each sector of society to support a nation of health and well-being, where each person is empowered to realize their full wellness potential.

Source: Health Care 2035 Advisory Panel (2015) The Japan Vision: Health Care 2035 Summary. http://www.mhlw.go.jp/healthcare2035

Issues and challenges

To ensure sustainability and equip the health care system to meet current and future health care needs, we need to:

- Break away from the patchwork style of health policy making
- Embrace comprehensive reform building upon a forward-looking and long-term vision.
- Transform health care into a social system that engages all sectors through shared vision and values, in contrast to maintaining the current system through basic cost share increases and benefits cuts.

Key questions

1. What existing policies should be strengthened?
2. What key issues have been repeatedly sidelined due to politics?
3. What should be discussed now to kick start future public debates?
4. How can Japan best contribute to other countries through health What can Japan learn from the health care experiences of other countries?

Paradigm shift

Existing model	Toward 2035
Quantity	Quality
Inputs	Value
Regulation	Autonomy
Cure	Care
Fragmentation	Integration

Overview of Health Care 2035

GOAL

A sustainable health care system that delivers unmatched health outcomes through care that is responsive and equitable to each member of society and that contributes to prosperity in Japan and around the world.

PRINCIPLES

- Fairness
- Solidarity built on autonomy
- Shared prosperity for Japan and the world

VISION

Key concepts for health care in 2035

1 LEAN HEALTHCARE

Implement value-based health care

2 LIFE DESIGN

Empower society and support personal choice

3 GLOBAL HEALTH LEADER

Lead and contribute to global health

INFRASTRUCTURE

Foundations of this vision

- Innovation
- Information
- Sustainable financing
- Health care professionals
- A world-class Ministry of Health, Labour and Welfare

3

Source: Health Care 2035 Advisory Panel (2015) The Japan Vision: Health Care 2035 Summary. http://www.mhlw.go.jp/healthcare2035

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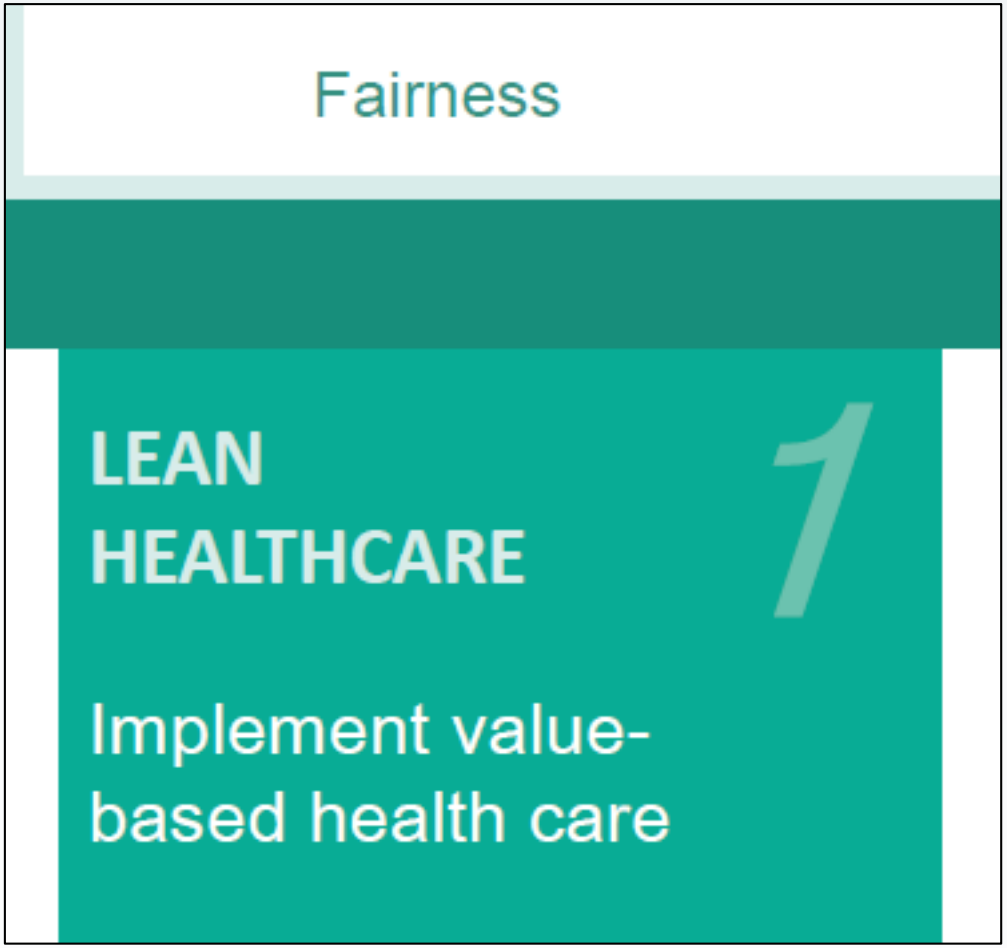

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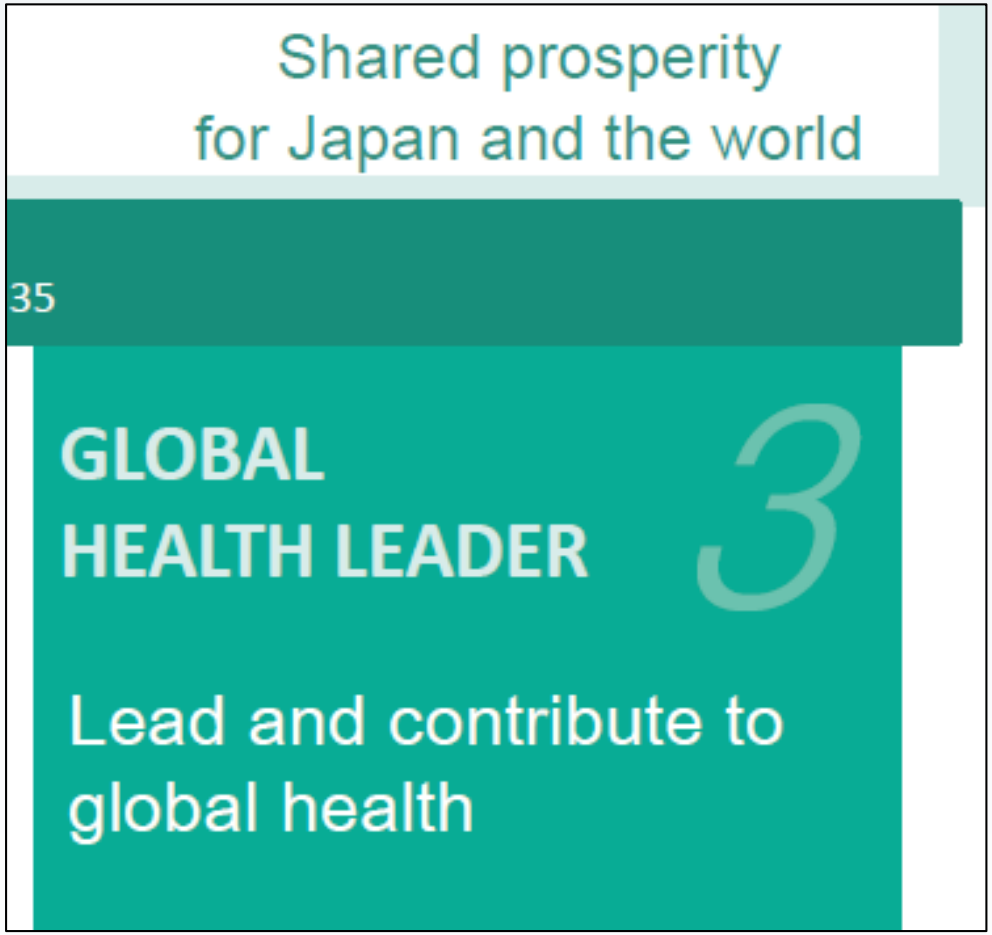

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Next step (1)

1. A headquarters was established within the Ministry of Health, Labour and Welfare to carry out the proposals, and, after careful consideration, steadily implement practical, short-term policies.
2. All 121 actions have been classified into three categories with a clear time line:
   - Do right now
   - Discuss and plan now
   - Propose alternatives
3. Five priority areas identified by Health Minister Yasuhisa Shiozaki: global health, ICT, health workforce, value-based payment, prevention

Next step (2)

"In making suggestions looking two decades ahead, some proposals and examples of policy will arouse debate. We welcome feedback and criticism on this report, and hope that it will be the start of national debate. We are sure that debate in itself will be the first step toward a brighter future."

Summary: future of Japan's health system

1. Japanese health system needs a new vision of health care and health systems for the future.
2. Health Care 2035 report proposes a paradigm shift for Japan's health system.
3. The new health system would maintain an emphasis on fairness and solidarity, while building on individual autonomy and engaging in global health policy-making.

Summary

1. Introduction
2. Health system performance
3. Population health
4. UHC: achievements and challenges
5. Cost containment
6. Service quality
7. Tackling aging population
8. Future directions